

Policy on Storage and Availability of Medications

«Hospital Name»

Document control

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Prepared by	Pharmacy In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

Medications at «Hospital Name» are stored safely and to manufacturer requirements everywhere in the hospital, including clinical areas. High-risk and look-alike/sound-alike drugs are kept physically apart, and emergency medications are always available and replenished immediately after use.

1. Purpose

This policy explains how «Hospital Name» stores medications safely, controls inventory, identifies and separates high-risk and look-alike/sound-alike medications, and keeps emergency medications available and replenished at all times.

This policy does not cover prescription, dispensing, or administration of medications in detail — those are covered in other hospital policies.

2. Scope

This policy applies to pharmacy staff and clinical staff involved in the storage of medications at «Hospital Name».

3. Policy standards

«Hospital Name» stores medications in a clean, safe, and secure environment that follows manufacturer recommendations everywhere in the hospital, including clinical areas, and

applies sound inventory control. A high-risk medication list is defined, kept current, and posted wherever high-risk medications are stored, and look-alike, sound-alike medications and different concentrations of the same drug are kept physically apart. A defined, uniformly stored emergency medication list is kept available at all times and replenished immediately after use.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

1. Do not store any medication outside its manufacturer's storage requirements, in any area of the hospital including clinical areas, and do not leave medications unprotected from loss or theft.
2. Do not check cold-storage temperature less often than once a day — or on every working day, for areas not open daily — and do not store expired medications alongside those intended for patient use.
3. Do not operate without a current, defined high-risk medication list posted in the pharmacy and every clinical area where high-risk medications are stored.
4. Do not store high-risk medications outside their predetermined, clinically justified locations, or without safeguards against inadvertent administration.
5. Do not store look-alike, sound-alike medications, or different concentrations of the same medication, physically together.
6. Do not leave an emergency medication location without the defined list, or leave a used emergency medication unreplenished.
7. Do not issue or use a medication that has been stored outside the manufacturer's temperature, light, or security recommendations until pharmacy has assessed it.
8. Do not store any other drug alongside emergency medications, or check emergency medication stock less often than daily — or, for sealed carts, less often than after each use or once a month.

5. What we do

5.1 Store medications safely

Medication storage spaces are clean, safe, and secure, following the manufacturer's storage requirements — where none exist, «Hospital Name» develops and applies its own. This applies to every area where medications are stored, including clinical areas. Medications are protected from loss or theft — for example by limiting access to authorised staff, locking medication carts and never leaving them unattended, or keeping them in a continuously staffed area. It is preferable for the storage area to be well organised, and overall cleanliness is maintained. Vaccines are kept at the manufacturer's required temperature, with temperature monitoring of the room or refrigerator done at least once a day — or on every working day, for areas not open daily. Medications past their expiry date are stored separately from those intended for patient use, pending disposal.

5.2 Apply sound inventory control

«Hospital Name» follows recognised inventory control practices — for example ABC analysis, VED (vital/essential/desirable), FSN (fast/slow/non-moving), or First-Expiry-First-Out analysis, alone or combined. Medicines could be stored alphabetically by generic name, and stock verification audits could run at intervals «Hospital Name» defines, to detect loss or theft. There is also a mechanism for handling medications outside the regular inventory — for example, physicians' samples not for sale.

5.3 Define and maintain the high-risk medication list

«Hospital Name» defines and periodically updates its list of high-risk medications — those carrying a heightened risk of serious harm if an error occurs, such as drugs with a low therapeutic window, controlled substances, psychotherapeutic medications, look-alike/sound-alike medications, and concentrated electrolytes. The list is available in the pharmacy and every clinical area where high-risk medications are stored.

5.4 Store high-risk medications where clinically necessary

High-risk medications are kept in predetermined areas — for example certain wards, the OT, or the ICU — based on clinical need. Where regulations apply, such as for narcotics, storage follows them. Safeguards are in place in every such area to prevent inadvertent administration.

5.5 Keep look-alike, sound-alike, and different-concentration medications apart

Look-alike and sound-alike medications, and different concentrations of the same drug, are identified periodically, drawn from the hospital formulary, and listed. This list is made available in every unit where drugs are stored, not just the pharmacy, and revised whenever the formulary or drug packaging changes. It is good practice to store these medications as far apart as possible — but at minimum, they are kept physically apart, in the pharmacy and in patient care areas alike.

5.6 Define and uniformly store emergency medications

The list of emergency medications is prepared in line with sound clinical practice and documented — it could vary by department, for example ICU, physiotherapy, emergency, or the cath lab. A crash cart with defined rows and drawers is a useful way to store these consistently. No other drug is kept with emergency medications.

5.7 Keep emergency medications available and replenished

Adequate quantities of emergency medications are stocked at all times, checked at least daily. Where «Hospital Name» uses a sealed emergency cart, the check happens after each use of the cart, or once a month, whichever comes first.

6. Stop-work authority

Do not store look-alike, sound-alike medications, or different concentrations of the same medication, physically together.

Do not leave an emergency medication location without the defined list, or leave a used emergency medication unreplenished.

Do not issue or use a medication that has been stored outside the manufacturer's temperature, light, or security recommendations until pharmacy has assessed it.

Stop-work applies to the storage location and to issue from that location. Immediate life-saving use of the only available dose continues while escalation happens, and is documented.

The person who stops tells the Pharmacy In-Charge and the Medication Safety Officer the same shift. Refusing unsafe storage or issue is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring medication storage requirements are resourced and implemented across the organisation.
Pharmacy In-Charge	Owens day-to-day implementation of this policy; maintains the high-risk and LASA lists; manages emergency medication replenishment; receives stop-work escalations the same shift.
Medication Safety Officer	Coordinates stop-work escalations; brings storage incidents and audit findings to the Drug and Therapeutics Committee.
Nursing Superintendent	Ensures clinical areas comply with manufacturer storage requirements, high-risk storage rules, and emergency medication check schedules.
Quality Coordinator	Audits this policy; holds training records and staff acknowledgements.

8. Quality monitoring

Monitoring area	What is monitored
Medication storage conditions	Manufacturer requirements followed in all areas including clinical areas; cold-storage temperature logged at least once a day or every working day; expired medications stored separately.
Inventory control	Recognised inventory control practices in use throughout the organisation.
High-risk medication list	Current, defined list updated periodically; posted in pharmacy and every clinical area that stores high-risk medications; staff trained.

High-risk storage locations	Predetermined locations documented; walk-round confirms high-risk medications stored only in authorised areas with safeguards.
LASA and different-concentration separation	List available in every unit where drugs are stored; physical separation confirmed in pharmacy and all patient care areas.
Emergency medication list and storage	Defined list documented; uniform storage layout; no other drugs stored alongside emergency medications.
Emergency medication availability	Daily inventory check at every location; sealed-cart check after each use or once a month; replenishment logged immediately after use.
Stop-work events	Stop-work events logged with trigger, action taken, and outcome.

9. Training and staff acknowledgement

Pharmacy staff and clinical staff involved in the storage of medications shall be familiar with the storage requirements, high-risk and LASA lists, emergency medication procedures, and the stop-work authority in this policy.

I have read the Policy on Storage and Availability of Medications of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to pharmacy staff, nursing staff, clinical staff involved in medication storage, the Pharmacy In-Charge, the Medication Safety Officer, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
ADR	Adverse drug reaction
DTC	Drug and Therapeutics Committee
FEFO	First expiry, first out
ICU	Intensive Care Unit
LASA	Look-alike, sound-alike
MOM	Management of Medication (NABH Hospitals chapter)

MSO	Medication Safety Officer
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OT	Operation Theatre

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
MOM.3.a	CORE	Sections 3 and 5.1 address clean, safe, secure storage following manufacturer requirements in all areas including clinical areas, protection from loss or theft, temperature monitoring at least once a day or every working day, and separation of expired medications.
MOM.3.b	Commitment	Section 5.2 addresses recognised inventory control practices throughout the organisation and a mechanism for non-regular-inventory medications.
MOM.3.c	CORE*	Sections 3 and 5.3 address the defined, periodically updated high-risk medication list posted in the pharmacy and every clinical area where high-risk medications are stored.
MOM.3.d	Achievement	Section 5.4 addresses storage of high-risk medications in predetermined, clinically justified locations with safeguards against inadvertent administration.
MOM.3.e	CORE*	Sections 3 and 5.5 address the identification, listing, and physical separation of LASA medications and different concentrations in the pharmacy and all

		patient care areas. Physical separation is also a stop-work trigger in Section 6.
MOM.3.f	Commitment*	Section 5.6 addresses the defined emergency-medication list prepared in line with sound clinical practice, documented, uniformly stored, and kept free of other drugs.
MOM.3.g	CORE	Section 5.7 addresses adequate emergency medication stock checked at least daily — or, for sealed carts, after each use or once a month — and replenished immediately after use.

13. Required Records/Evidence Checklist

Medication storage conditions — MOM.3.a (CORE)

1. Storage-condition records showing manufacturer requirements followed in all areas of the hospital, including clinical areas.
2. Cold-storage and refrigerator temperature log with entries at least once a day — or on every working day for areas not open daily — and excursion-reporting records.
3. Access-control records for the main pharmacy and controlled-drug cupboards.
4. Records showing expired medications stored separately from those intended for patient use, pending disposal.

Inventory control — MOM.3.b

1. Documented inventory-control method (for example FEFO, ABC, VED, or FSN) in use throughout the organisation.
2. Stock-movement traceability records from receipt to issue for sampled items.

High-risk medication list — MOM.3.c (CORE*)

1. Current, DTC-approved high-risk medication list, updated periodically and after any related incident.
2. Posted-list records showing the current list available in the pharmacy and every clinical area where high-risk medications are stored.
3. Staff training records on the high-risk list.

High-risk storage locations — MOM.3.d

1. DTC record naming the predetermined locations where each high-risk medication is clinically necessary.
2. Walk-round record confirming high-risk medications stored only in authorised areas.

3. Safeguard documentation in each designated high-risk storage area.

LASA and different-concentration separation — MOM.3.e (CORE*)

1. LASA and different-concentration medication list developed from the hospital formulary, available in every unit where drugs are stored.
2. Physical-separation records showing LASA medications and different concentrations stored apart — in separate bins or on separate shelves — in the pharmacy and all patient care areas.
3. List-revision records following formulary changes or drug packaging changes.

Emergency medication list and storage — MOM.3.f (Commitment*)

1. Defined, documented emergency-medication list prepared in line with sound clinical practice.
2. Uniform-storage layout record across crash carts and emergency trolleys.
3. Confirmation that no other drug is stored alongside emergency medications.

Emergency medication availability and replenishment — MOM.3.g (CORE)

1. Daily inventory-check records at every emergency-medication location (unsealed cart).
2. Sealed-cart check records: after each use of the cart, or once a month, whichever comes first.
3. Immediate-replenishment log after any use of an emergency medication, before the cart is returned to service.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. MOM.3.

Disclaimer

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